



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

QUESTIONS

Q1. A 26-year-old woman presents to the emergency department with complaints of progressive fatigue, joint pain, and swelling in her hands and knees for the past 4 months. She also reports intermittent low-grade fever and hair loss. Over the last few weeks, she has noticed a rash across her cheeks that becomes more prominent after sun exposure. She also gives a history of painless oral ulcers and recent swelling of her legs.

On examination, she appears pale. There is a well-demarcated erythematous rash over the malar area sparing the nasolabial folds. Bilateral pedal edema is present. Musculoskeletal examination reveals tenderness and mild swelling of the metacarpophalangeal and proximal interphalangeal joints.

Which of the following antibodies is most specific for confirming the diagnosis in this patient?

- a) Anti-double stranded DNA (anti-dsDNA) antibody
- b) Anti-Smith (anti-Sm) antibody
- c) Anti-histone antibody
- d) Anti-centromere antibody

Q2. A 50-year-old male presents with progressive breathlessness and dry cough for the past 2 years. He also complains of tightness of skin over his fingers, difficulty swallowing, and episodes of finger discoloration on exposure to cold. He has worked in a sandstone quarry for over 25 years with significant exposure to silica dust. On examination, there is sclerodactyly, digital ulcers, and reduced oral aperture. Chest auscultation reveals fine bibasilar crackles.

Investigations reveal:

- Chest X-ray: Multiple nodular opacities predominantly in the upper lobes
- HRCT chest: Interstitial fibrosis
- Pulmonary function test: Restrictive pattern

Which of the following combinations best establishes the diagnosis of Erasmus syndrome in this patient?

- a) Silica exposure + pulmonary nodules + Anti-double stranded DNA positivity
- b) Silica exposure + interstitial lung disease + Anti-topoisomerase I (anti-Scl-70) positivity
- c) Silica exposure + cavitory lung lesions + Anti-centromere antibody positivity
- d) Silica exposure + pleural effusion + Anti-histone antibody positivity

Q3. A 42-year-old woman presents with progressive tightening of the skin over her hands and face for the past 1 year. She reports difficulty in making a fist and has noticed that her fingers turn white and then bluish on exposure to cold. She also complains of difficulty swallowing solid foods and frequent episodes of heartburn.

On examination, there is shiny, thickened skin over the fingers (sclerodactyly), reduced oral aperture, and multiple telangiectasias over the face. Digital pitting scars are noted at the fingertips. Investigations reveal: ANA: Positive, Barium swallow: Esophageal dysmotility, HRCT chest: Early interstitial lung changes.

Which of the following is the most likely diagnosis?

- a) Systemic sclerosis (scleroderma)
- b) Rheumatoid arthritis

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

- c) Systemic lupus erythematosus
- d) Dermatomyositis

Q4. Identify the image:



- a) Swan Neck Deformity; DIP- Extension PIP- Flexion
- b) Boutonniere Deformity; DIP- Extension PIP- Flexion
- c) Swan Neck Deformity; DIP- Flexion PIP- Extension
- d) Boutonniere Deformity; DIP- Flexion PIP- Extension

Q5. A 26-year-old woman presents with fever, weight loss, oral ulcers, pleuritic chest pain, and hair loss. She has intermittent photosensitive rash. Labs: Hb 8.9 g/dL, platelets 85,000, WBC 2,800. Urinalysis: RBC casts, proteinuria 1.8 g/day. ANA 1:640 (homogeneous), anti-dsDNA high, C3/C4 low. ECG normal.

What is the MOST likely classification of her kidney disease?

- a) Class II mesangial
- b) Class III focal
- c) Class IV diffuse
- d) Class V membranous

Q6. A 67-year-old man presents with a long-standing history of joint pain involving the midfoot and knees. The pain has been progressive over several years and is associated with intermittent swelling. He also reports a past history of acute episodes of severe joint pain in the big toe that resolved spontaneously.

On examination, there are firm, non-tender, yellowish nodules over the extensor surface of the elbows and around the joints. The affected joints show bony deformities and reduced range of motion. Investigations reveal: Serum uric acid: Elevated, X-ray of affected joints: "Punched-out" erosions with sclerotic margins and overhanging edges.

Which of the following is the most likely diagnosis?



- a) Pseudogout



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

- b) Chronic tophaceous gout
- c) Rheumatoid arthritis
- d) Septic arthritis

Q7.A 24-year-old man is evaluated for lifelong mild jaundice that worsens during fasting or illness. He has no abdominal pain, pruritus, or weight loss. Physical examination is unremarkable other than scleral icterus. Liver is smooth and normal in color.

Laboratory results:

Total bilirubin: 3.8 mg/dL

Direct (conjugated) bilirubin: elevated

AST/ALT: normal

ALP: normal

Urine coproporphyrin analysis shows increased type I coproporphyrin.

Ultrasound of the liver is normal.

Which diagnosis best explains his presentation?

- a) Rotor syndrome
- b) Dubin-Johnson syndrome
- c) Gilbert syndrome
- d) Autoimmune hepatitis

Q8.A 27-year-old man presents with fatigue, nausea, right upper quadrant discomfort, and dark urine for 5 days. He recently had unprotected sexual contact.

Labs show: ALT 950 U/L, AST 720 U/L, total bilirubin 3.1 mg/dL.

Hepatitis B serology:**

HBsAg: Positive

Anti-HBc IgM: Positive

HBeAg: Positive

Anti-HBs: Negative

What is the most likely diagnosis?

- a) Acute Hepatitis B infection
- b) Chronic active Hepatitis B
- c) Window period of Hepatitis B
- d) Immunity due to vaccination

Q9.A 45-year-old man with no symptoms undergoes a routine medical check. He is a migrant worker from an



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MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

HBV-endemic region.

Serology:**

HBsAg: Positive for >6 months

Anti-HBc IgG: Positive

HBeAg: Positive

HBV DNA: High

Anti-HBs: Negative

LFTs: ALT mildly elevated.

What is the correct interpretation?

- a) Past infection with immunity
- b) Vaccination-induced immunity
- c) Chronic Hepatitis B with high viral replication
- d) Acute resolving Hepatitis B

Q 10. A 33-year-old woman presents with malaise and mild jaundice. She reports flu-like symptoms 3 weeks earlier.

Hepatitis B serology:**

HBsAg: Negative

Anti-HBs: Negative

Anti-HBc IgM: Positive

HBeAg: Negative

What is the most likely explanation for this pattern?

- a) Past resolved infection
- b) Vaccinated, no infection
- c) Window period of acute HBV
- d) Chronic inactive carrier state

Q11. A 28-year-old woman presents with progressive fatigue, malaise, and intermittent jaundice for the past 3 months. She also reports anorexia and mild right upper quadrant discomfort. There is no history of alcohol use, blood transfusion, or high-risk behavior. She has a past history of autoimmune thyroid disease. On examination, she appears icteric. Mild hepatomegaly is noted, and there are no signs of chronic liver failure.

Laboratory investigations reveal:

- AST: 420 IU/L
- ALT: 510 IU/L
- ALP: Mildly elevated
- Total bilirubin: 3.2 mg/dL
- Serum IgG: Elevated



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

- Viral hepatitis markers (HBsAg, anti-HCV): Negative

Further immunological testing is planned to confirm the diagnosis.

Which of the following is the most specific antibody associated with this condition?

- a) Anti-mitochondrial antibody
- b) Anti-smooth muscle antibody
- c) Anti-liver kidney microsomal (anti-LKM-1) antibody
- d) Anti-centromere antibody

Q12. A 58-year-old man with a history of chronic alcohol use presents with abdominal distension, jaundice, and altered sensorium. On examination, he has ascites and asterixis. Laboratory investigations show elevated bilirubin, low albumin, and prolonged INR. The physician decides to assess the severity of his chronic liver disease using the Child–Pugh scoring system.

Which of the following set of parameters is included in the Child–Pugh score?

- a) Serum bilirubin, serum albumin, INR, ascites, hepatic encephalopathy
- b) Serum bilirubin, ALT, AST, platelet count, ascites
- c) Serum albumin, INR, creatinine, ascites, portal vein diameter
- d) Bilirubin, alkaline phosphatase, prothrombin time, splenomegaly, encephalopathy

Q13. A 17-year-old boy is brought by his parents due to progressive mood changes, irritability, and declining academic performance over the past year. His teachers report new-onset difficulty with concentration and episodes of inappropriate laughter. Over the past 3 months, he has also developed slurred speech, difficulty writing, and occasional tremors in both hands, especially when reaching for objects.

He denies alcohol or drug use. There is no family history of neurologic disorders.

On physical examination, he appears mildly jaundiced. His abdomen is soft but shows palpable hepatomegaly. Neurologic examination reveals dysarthria, a wing-beating tremor, and mild rigidity of the upper limbs.

Routine labs show:

AST: 118 U/L

ALT: 102 U/L

Bilirubin mildly elevated

INR slightly prolonged

Further testing reveals:

Serum ceruloplasmin: significantly low

24-hour urinary copper: markedly elevated

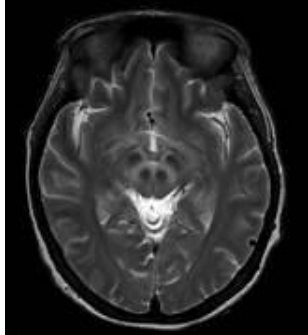
A slit-lamp eye examination demonstrates golden-brown rings encircling the cornea.

Which diagnosis best explains this patient's presentation?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026



- a) Early-onset Huntington disease
- b) Wilson disease
- c) Autoimmune hepatitis
- d) Hereditary hemochromatosis

Q14. A 52-year-old man presents with progressive fatigue, decreased libido, and intermittent pain in the second and third MCP joints of both hands. He reports that his “skin seems to be getting darker,” even though he has not been spending much time in the sun. Over the last year, he also developed poor glycemic control, requiring escalation of diabetes medications.

He denies alcohol use and has no history of blood transfusions. On examination, his skin appears bronze-colored, and he has hepatomegaly without tenderness. Testicular size is slightly reduced.

Laboratory results show:

AST: 92 U/L

ALT: 88 U/L

Fasting glucose: elevated

Serum ferritin: markedly elevated

Transferrin saturation: very high (= 70%)

HbA1c: elevated

Genetic testing reveals C282Y homozygosity in the HFE gene.

Which diagnosis best fits this presentation?

- a) Wilson disease
- b) Alcoholic liver disease
- c) Hereditary hemochromatosis
- d) Autoimmune hepatitis

Q15. A 32-year-old woman is brought to the emergency department with altered sensorium for 2 days. She had a history of jaundice, nausea, and vomiting for the past 1 week. There is no prior history of liver disease. On examination, she is disoriented and has asterixis. Icterus is present. There are no stigmata of chronic liver disease.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

Laboratory investigations reveal:

- Markedly elevated AST and ALT
- Prolonged INR
- Low blood glucose levels

A diagnosis of fulminant hepatitis is made.

Which of the following statements regarding the complications of her condition is NOT true?

- a) Hepatic encephalopathy occurs due to accumulation of ammonia and other neurotoxins
- b) Cerebral edema is a major cause of mortality in this condition
- c) Hypoglycemia occurs due to impaired gluconeogenesis
- d) Portal hypertension and esophageal varices typically develop early in the disease

Q16. A 25-year-old woman presents with recurrent episodes of fatigue and muscle weakness for the past several months. She also reports a history of multiple kidney stones. There is no history of diarrhea or diuretic use. On examination, she appears mildly dehydrated. Blood pressure is normal.

Laboratory investigations reveal:

- Serum potassium: 3.0 mEq/L
- Serum bicarbonate: 14 mEq/L
- Arterial blood gas: Metabolic acidosis with normal anion gap
- Urine pH: 6.5

Ultrasound of the kidneys shows nephrocalcinosis.

Which of the following is the most likely diagnosis?

- a) Type 1 (distal) renal tubular acidosis
- b) Type 2 (proximal) renal tubular acidosis
- c) Type 4 renal tubular acidosis
- d) Chronic kidney disease

Q17. A 63-year-old man with long-standing hypertension presents with poor blood pressure control despite being on three antihypertensive medications, including a calcium channel blocker, ACE inhibitor, and diuretic. He also reports episodes of flash pulmonary edema.

On examination, his blood pressure is 180/100 mmHg. An abdominal bruit is heard on auscultation.

Laboratory investigations reveal:

- Serum creatinine: Mildly elevated
- Potassium: Normal

Doppler ultrasound suggests reduced renal blood flow. Further evaluation with CT angiography reveals significant narrowing of the renal artery involving distal branches rather than the main proximal artery.

Which of the following is the most appropriate management for this patient?

- a) Percutaneous transluminal angioplasty with stenting
- b) Surgical revascularization (bypass surgery)
- c) Continue optimal medical therapy alone



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

d) Immediate nephrectomy

Q18. A 68-year-old man is brought to the emergency department with complaints of decreased urine output and dizziness for 2 days. He has had multiple episodes of vomiting and poor oral intake over the past week. He has a history of hypertension and is on diuretics.

On examination, he appears dehydrated with dry mucous membranes and hypotension (BP: 90/60 mmHg).

Laboratory investigations reveal elevated serum creatinine, and acute kidney injury is suspected.

Which of the following laboratory findings is most consistent with prerenal acute kidney injury?

- a) BUN/Creatinine ratio $<10:1$, urine sodium >40 mEq/L, FENa $>2\%$
- b) BUN/Creatinine ratio $>20:1$, urine sodium <20 mEq/L, FENa $<1\%$
- c) BUN/Creatinine ratio $10-15:1$, urine sodium >40 mEq/L, FENa $>2\%$
- d) BUN/Creatinine ratio $<15:1$, urine sodium <20 mEq/L, FENa $>2\%$

Q19. A 55-year-old man with a known history of chronic kidney disease presents to the emergency department with complaints of generalized weakness and palpitations for the past few hours. He has missed his last dialysis session. On examination, his pulse is irregular, and blood pressure is 140/90 mmHg.

Laboratory investigations reveal: Serum potassium: 6.2 mEq/L

An ECG is performed, which shows tall, peaked T waves.

The patient is at risk of developing life-threatening cardiac arrhythmias.

What is the first step in management for this patient?

- a) IV insulin with glucose
- b) IV calcium gluconate
- c) Furosemide
- d) Sodium bicarbonate

Q20. A 5-year-old boy is brought to the pediatric clinic with swelling around the eyes for the past 1 week, which is more prominent in the morning. His parents also noticed swelling of his legs and a recent decrease in urine output. There is no history of hematuria, fever, or recent sore throat.

On examination, he has periorbital edema, pedal edema, and ascites. Blood pressure is normal.

Laboratory investigations reveal:

- Urinalysis: Proteinuria (++++), with no RBCs
- Serum albumin: 2.0 g/dL
- Serum cholesterol: Elevated
- Serum creatinine: Normal

A renal biopsy is considered.

Which of the following findings is most characteristic of this condition?

- a) Effacement of podocyte foot processes on electron microscopy
- b) Thickened glomerular basement membrane with subepithelial deposits



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

- c) Mesangial proliferation with IgA deposition
- d) Linear IgG deposition along the basement membrane

Q21. A 48-year-old man presents with progressive swelling of the legs and frothy urine for the past 2 months. He denies hematuria, fever, or recent infections. He has no history of diabetes or hypertension. On examination, he has bilateral pitting pedal edema and mild ascites. Blood pressure is 130/80 mmHg.

Laboratory investigations reveal:

- Urinalysis: Proteinuria (++++)
 - 24-hour urinary protein: 5.5 g/day
- Serum albumin: 2.3 g/dL
- Serum cholesterol: Elevated
- Serum creatinine: Normal

A renal biopsy shows thickening of the glomerular basement membrane with subepithelial immune complex deposition ("spike and dome" appearance).

Which of the following antibodies is most commonly associated with this condition?

- a) Anti-phospholipase A2 receptor (anti-PLA2R) antibody
- b) Anti-double stranded DNA antibody
- c) Anti-glomerular basement membrane antibody
- d) Anti-neutrophil cytoplasmic antibody

Q22. A 52-year-old man with a 10-year history of poorly controlled diabetes mellitus presents with gradually increasing pedal edema. He also reports frothy urine. On examination, he has bilateral pitting edema. Blood pressure is 150/95 mmHg. Laboratory investigations reveal persistent proteinuria and elevated HbA1c. A renal biopsy is performed.

Which of the following is the most characteristic histopathological finding in this condition?

- a) Crescent formation in glomeruli
- b) Subepithelial immune complex deposition
- c) Effacement of podocyte foot processes only
- d) Nodular mesangial expansion (Kimmelstiel–Wilson nodules)

Q23. A 12-year-old boy presents with cola-colored urine. His mother reports he had a sore throat 2 days ago.

Vital signs: mild hypertension.

Labs:

Urinalysis: red blood cell casts

Complement C3: normal

Serum creatinine: mild elevation

Renal biopsy shows mesangial proliferation



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

What is the most likely diagnosis?

- a) Post-streptococcal glomerulonephritis (PSGN)
- b) IgA nephropathy
- c) Membranoproliferative GN
- d) Henoch–Schönlein purpura

Q24. A 7-year-old boy is brought to the clinic with swelling around the eyes and cola-colored urine for 5 days. His mother reports that he had a sore throat about 2 weeks ago.

On examination, he has periorbital edema and mild hypertension.

Laboratory investigations reveal:

- Urinalysis: Hematuria with RBC casts
- Serum complement (C3): Decreased

A renal biopsy is performed.

Which of the following findings on electron microscopy is most characteristic of this condition?

- a) Effacement of podocyte foot processes
- b) Subendothelial immune complex deposition
- c) Subepithelial “hump-like” electron-dense deposits
- d) Linear IgG deposition along the basement membrane

Q25. A 14-year-old boy presents with recurrent episodes of hematuria since early childhood. He also reports progressive hearing difficulty. His maternal uncle had a history of kidney failure at a young age.

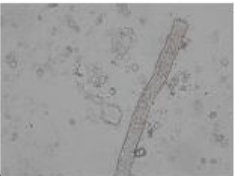
On examination, there is no edema. Audiometry confirms sensorineural hearing loss. Laboratory investigations reveal persistent microscopic hematuria with mild proteinuria. Renal biopsy shows irregular thickening and splitting of the glomerular basement membrane. Which of the following abnormalities is most likely responsible for this condition?

- a) Defect in type IV collagen $\alpha 5$ chains
- b) Defect in type III collagen $\alpha 2$ chain
- c) Defect in type IV collagen $\alpha 1$ chain
- d) Defect in type II collagen $\alpha 5$ chains



MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

Q26. Match the casts with the conditions they are found in:

Casts	Condition
1- 	a- Nephritic syndrome
2- 	b- CKD
3- 	c- ATN
4- 	d- PN

- a) 1-d, 2-c, 3-a, 4-b
- b) 1-a, 2- d, 3- b, 4- c
- c) 1-d, 2- a, 3- c, 4- b
- d) 1-a, 2- d, 3-c, 4-b

Q27. A 21-year-old man presents with persistent hypokalemia, metabolic alkalosis, hypertension, and low renin/low aldosterone. Diagnosis?

- a) Bartter syndrome
- b) Gitelman syndrome
- c) Liddle syndrome
- d) Conn syndrome

Q28. A 13-year-old boy is brought by his parents due to excessive increase in height over the past 2 years. He is significantly taller than his peers and has enlarged hands and feet. His school reports difficulty in fitting into desks due to his rapid growth. There is no history of chronic illness. On examination, his height is above the 97th percentile for age. He has coarse facial features and enlarged extremities. A growth hormone-secreting pituitary adenoma is suspected.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

Which of the following is the most appropriate investigation of choice to confirm the diagnosis?

- a) Serum IGF-1 levels
- b) MRI of the pituitary gland
- c) Oral glucose tolerance test (OGTT)
- d) Random growth hormone levels

Q29. A 6-year-old boy is brought to the clinic with excessive thirst and passage of large volumes of urine since early childhood. His parents report that he drinks water frequently, even at night. There is no history of diabetes mellitus.

On examination, he appears mildly dehydrated. Serum osmolality: Increased Urine osmolality: Low A water deprivation test is performed, followed by administration of desmopressin. There is no significant increase in urine osmolality after desmopressin. Which of the following is the most likely diagnosis?

- a) Central diabetes insipidus
- b) Nephrogenic diabetes insipidus
- c) Primary polydipsia
- d) Syndrome of inappropriate ADH secretion

Q30. A 45-year-old man with central obesity, moon face, and proximal muscle weakness has high serum cortisol not suppressed by low-dose dexamethasone. ACTH is high. Which hormone overproduction is most likely?

- a) Cortisol from adrenal medulla
- b) ACTH from the pituitary
- c) Renin from kidneys
- d) CRH from hypothalamus

Q31. A 36-year-old woman presents with progressive fatigue, weight gain, and intolerance to cold for the past 6 months. She also complains of constipation and hair thinning. There is no history of thyroid surgery or radiation exposure.

On examination, she has dry skin, bradycardia, and a diffusely enlarged, non-tender thyroid gland.

Laboratory investigations reveal:

- TSH: Elevated
- Free T4: Decreased

Which of the following antibodies is most commonly associated with this condition?

- a) Anti-TSH receptor antibody
- b) Anti-thyroid peroxidase (anti-TPO) antibody
- c) Anti-thyroglobulin receptor antibody
- d) Anti-centromere antibody

Q32. A 29-year-old woman presents with complaints of weight loss despite increased appetite, palpitations, heat intolerance, and excessive sweating for the past 3 months. She also reports increased irritability and



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MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

difficulty sleeping. On examination, her pulse is 110/min and irregular. She has a diffuse, non-tender enlargement of the thyroid gland. Prominent eye findings include bilateral exophthalmos and lid lag. Fine tremors are noted in her hands.

Laboratory investigations reveal:

- TSH: Suppressed
- Free T4: Elevated

Which of the following is the most specific antibody responsible for this condition?

- a) Anti-thyroid peroxidase antibody
- b) Anti-TSH receptor (thyroid-stimulating immunoglobulin) antibody
- c) Anti-thyroglobulin antibody
- d) Anti-microsomal antibody

Q33. A 58-year-old man with a 10-year history of chronic kidney disease presents with generalized bone pain and proximal muscle weakness for the past several months. He also reports pruritus and fatigue. He is on conservative management and has not yet started dialysis.

On examination, there is diffuse bone tenderness. No neck swelling is noted.

Laboratory investigations reveal: Serum calcium: Low, Serum phosphate: Elevated, Parathyroid hormone (PTH): Markedly elevated, Vitamin D: Decreased

Which of the following is the most likely diagnosis?

- a) Primary hyperparathyroidism
- b) Secondary hyperparathyroidism
- c) Tertiary hyperparathyroidism
- d) Hypoparathyroidism

Q34. 42-year-old man presents with long-standing hypertension that is poorly controlled despite multiple medications. He also complains of intermittent muscle weakness and cramps.

On examination, his blood pressure is 170/100 mmHg.

Laboratory investigations reveal: Elevated plasma aldosterone–renin ratio

Primary hyperaldosteronism (Conn syndrome) is suspected.

Which of the following is the investigation of choice to confirm the diagnosis?

- a) Plasma aldosterone–renin ratio
- b) CT scan of adrenal glands
- c) Saline infusion test
- d) 24-hour urinary metanephrines

Q35. A 36-year-old woman presents with recurrent episodes of severe headache, palpitations, and excessive sweating for the past 6 months. These episodes are sudden in onset and last for several minutes. She also reports anxiety during these attacks. On examination, her blood pressure is 180/110 mmHg during an episode,



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

which returns to near normal between episodes. Which of the following is the investigation of choice to establish the diagnosis?

- a) CT scan of the abdomen
- b) Serum cortisol levels
- c) Fractionated plasma or urinary metanephrines
- d) 24 hrs VMA

Q36. A 38-year-old woman presents with weight gain, moon face, proximal muscle weakness, and purple abdominal striae. On examination, she is hypertensive. Screening tests suggest hypercortisolism.



Which of the following is the confirmatory test for this condition?

- a) High dose dexamethasone suppression test (24 mg)
- b) 24-hour urinary free cortisol
- c) Late-night salivary cortisol
- d) Low-dose dexamethasone suppression test (48-hour)

Q37. A 45-year-old man presents with recurrent episodes of sweating, palpitations, confusion, and dizziness, especially when he skips meals. During one such episode in the hospital, his blood glucose is found to be 42 mg/dL. His symptoms improve rapidly after administration of intravenous glucose. Which of the following best describes the diagnostic criteria demonstrated in this patient?

- a) Virchow's triad
- b) Beck's triad
- c) Whipple's triad
- d) Charcot's triad

Q38. A 34-year-old woman presents with recurrent kidney stones. Evaluation reveals parathyroid adenoma causing hypercalcemia. Further workup shows a pituitary adenoma (prolactinoma). Genetic testing for MEN1 mutation is negative. Which of the following is the most likely diagnosis?

- a) MEN type 1
- b) MEN type 2A
- c) MEN type 2B
- d) MEN type 4

Q39. A 19-year-old woman with a known history of type 1 diabetes mellitus is brought to the emergency department with vomiting, abdominal pain, and altered sensorium for 1 day. She has missed her insulin doses



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

for the past few days.

On examination, she is dehydrated, tachycardic, and has deep, rapid breathing. Her breath has a fruity odor.

Laboratory investigations reveal:

- Blood glucose: 420 mg/dL
- Arterial pH: 7.18
- Serum bicarbonate: 10 mEq/L
- Serum ketones: Positive

Which of the following best explains the underlying mechanism of this condition?

- a) Increased insulin leading to hypoglycemia
- b) Increased lipolysis leading to ketone body formation
- c) Decreased glucagon secretion
- d) Increased renal excretion of glucose

Q40. A 17-year-old boy is evaluated for mild hyperglycemia detected during a routine health check. He is asymptomatic, has a normal BMI, and no signs of insulin resistance. His father and grandfather were diagnosed with diabetes at a young age and are managed without insulin.

Laboratory investigations show:

- Fasting glucose: Mildly elevated
- HbA1c: 6.8%
- C-peptide: Normal
- Autoantibodies for type 1 diabetes: Negative

A monogenic form of diabetes is suspected.

Which of the following gene defects is most commonly associated with this condition?

- a) HNF1A gene mutation
- b) Insulin receptor gene mutation
- c) GLUT4 gene mutation
- d) Leptin gene mutation

ANSWERS

1	b
2	b
3	a
4	c
5	c
6	b
7	b
8	a

9	c
10	c
11	b
12	a
13	b
14	c
15	d
16	a

17	a
18	b
19	b
20	a
21	a
22	d
23	b
24	c



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST DATED ON 11 JUNE 2026

25	a
26	c
27	c
28	c
29	b
30	b

31	b
32	b
33	b
34	c
35	c
36	d

37	c
38	d
39	b
40	a

